A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Dejan Fiser
Medical One

Dear Dr Fiser,

Re: Ms Shi Cheah
DOB: 1995-08-30
Address: 106/145 Queensberry Street, CARLTON, VIC, 3053

Summary

- **Clinical improvement in gastrointestinal stability on increased nortriptyline.**
- **Recurrence of migraines following the reduction of carbamazepine.**
- **Significant psychosocial stressors contributing to clinical presentation.**
- **No action required by you.**

I had the pleasure of conducting a review with Belle. As you may be aware, Belle has faced significant psychosocial challenges recently, including a crisis involving domestic stressors. She is currently managing a difficult home situation and is in the process of engaging with support systems and her siblings to ensure her safety.

From a gastroenterological perspective, her symptoms have shown relative improvement compared to the preceding months. She has increased her nortriptyline to 25 mg at night and has reduced her carbamazepine to 300 mg as previously suggested. While the transition was initially difficult, she reports fewer prolonged vomiting episodes. However, she did note a recent increase in migraines, which she attributes to the carbamazepine dose reduction.

Belle reported a new, albeit infrequent, symptom of coughing during meals followed by the regurgitation of undigested food. This has occurred twice recently. Additionally, she continues to experience a specific pattern of chest pain associated with the urge to defaecate. This pain is often accompanied by diaphoresis and resolves immediately following bowel action. Her bowel habits remain variable, ranging from normal stools to episodes of watery diarrhoea, sometimes occurring post-prandially.

All appointments and enquires
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Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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Impression and Plan The symptoms of chest pain associated with bowel movements likely represent visceral hypersensitivity and referred pain triggered by colonic distension. While nortriptyline generally mitigates such signals, the significant psychological stress Belle is currently enduring is undoubtedly exacerbating her physiological symptoms. We discussed the possibility of trial medications such as nizatidine for her chest discomfort, but Belle prefers not to add further medications at this stage.

I have encouraged her to maintain her current regimen of nortriptyline 25 mg at night, domperidone 10 mg three times daily, and esomeprazole 20 mg daily. I will assist in facilitating a priority follow-up with her psychiatrist, Hannah, whose clinics have been recently affected by family emergencies. I have provided Belle with scripts for nortriptyline and domperidone. We shall review her progress again in four months, although she is aware she may contact me sooner if required.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Functional Gut Clinic

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